PTSD and Trauma-Related Disorders (DSM-5-TR): Teaching Overview

Clinical Companion Knowledge Library: for counselors and counseling students

1. What it is

Posttraumatic stress disorder (PTSD) is a lasting reaction to a traumatic event in which the memory keeps intruding, the person avoids reminders, beliefs about self and world turn negative, and the body stays on alert. In DSM-5-TR it sits in "Trauma- and Stressor-Related Disorders," with acute stress disorder and adjustment disorders (American Psychiatric Association [APA], 2022).

How common: across 24 countries, 70.4% of adults had experienced at least one trauma, and rape, other sexual assault, stalking, and the unexpected death of a loved one carried the largest share of the PTSD burden (Kessler et al., 2017). In a U.S. sample, 89.7% reported a DSM-5 traumatic event and lifetime PTSD was 8.3%, higher in women (Kilpatrick et al., 2013).

Who develops it: risk factors each have modest effects. Factors during and after the trauma, such as its severity, **lack of social support**, and added life stress, predicted PTSD more strongly than factors before it (Brewin et al., 2000).

Counselor framing: most people exposed to trauma do not develop PTSD. For those who do, trauma-focused therapy is the first-line treatment, and a counselor trained in it can do much of the work.

2. Core symptoms to watch for

DSM-5-TR criteria for adults, paraphrased (APA, 2022); use the manual itself for formal diagnosis.

- **Exposure** to actual or threatened death, serious injury, or sexual violence: directly, as a witness, by learning it happened to someone close (if violent or accidental), or by repeated exposure to grim details at work.
- **Intrusion** (1 or more): unwanted memories, nightmares, flashbacks, distress at reminders, or strong body reactions to reminders.
- **Avoidance** (1 or more) of trauma-related thoughts and feelings or of outside reminders.
- **Negative changes in thinking and mood** (2 or more), including exaggerated negative beliefs ("I am bad," "no one can be trusted"), distorted blame of self or others, persistent fear, guilt, or shame, loss of interest, feeling cut off from others, and inability to feel positive emotions.
- **Arousal and reactivity** (2 or more): irritability or anger, reckless behavior, hypervigilance, exaggerated startle, poor concentration, and sleep problems.
- **Duration and impact:** more than 1 month, with significant distress or impairment, not due to a substance or medical condition. Specifiers: with dissociative symptoms; with delayed expression (full criteria 6 or more months after the event).

Related diagnoses (APA, 2022): *acute stress disorder* (3 days to 1 month after the trauma); *adjustment disorder* (a stressor that need not be traumatic, with milder symptoms); *prolonged grief disorder* (see its own review). Children 6 and under have separate criteria.

Screening tools: the 5-item PC-PTSD-5 showed excellent accuracy in primary care (AUC 0.94); a cut score of 3 maximized sensitivity (Prins et al., 2016). The 20-item PCL-5 tracks symptoms over time and showed strong reliability ($\alpha = .94$, test-retest $r = .82$) (Blevins et al., 2015).

Suicide risk: across 50 studies, PTSD was linked to past suicide attempts and to past and current suicidal thinking, partly through co-occurring depression (Krysinska & Lester, 2010). Ask directly.

When counselors must refer (general clinical consensus): refer for psychiatric evaluation for suicidality, psychosis, severe dissociation, or substance dependence. Address current danger (for example, ongoing domestic violence) before trauma processing.

3. What works: treatment and the evidence

Bottom line for class: trauma-focused CBT is the first-line treatment and outperforms medication over the long term. Its effects are large, but many clients still have symptoms afterward.

Trauma-focused CBT (honest effect sizes):

- **Breadth of evidence:** across 114 trials (8,171 people), trauma-focused CBT and EMDR had clinically important effects; Cognitive Processing Therapy (CPT), Cognitive Therapy, and Prolonged Exposure had the strongest evidence (Lewis et al., 2020). In 64 trials, effects were large (d about 1.0) and fewer than 4 people needed treatment for one to lose the diagnosis (Cusack et al., 2016).
- **CPT and exposure:** among 171 women after rape, CPT and prolonged exposure were both highly effective and better than minimal attention (Resick et al., 2002).
- **Cognitive therapy for recent PTSD:** at follow-up, 11% still had PTSD after cognitive therapy versus 61% after a self-help booklet (Ehlers et al., 2003).
- **Realistic expectations:** in military PTSD, 49% to 70% improved meaningfully with CPT or prolonged exposure, but about two-thirds still met diagnostic criteria afterward (Steenkamp et al., 2015).

Medication (referral notes). SSRIs beat placebo, but the effect was small (SMD −0.28); fluoxetine, paroxetine, sertraline, and venlafaxine had support (Hoskins et al., 2021). At long-term follow-up, psychotherapy beat medication (SMD −0.83) (Merz et al., 2019).

What to avoid: single-session psychological debriefing right after a trauma did not prevent PTSD, and one trial found higher PTSD risk a year later (OR 2.88) (Rose et al., 2002).

CBT for PTSD: how it is used. PTSD persists when the trauma is appraised in excessively negative ways and the memory is poorly processed, so the person feels a *current* threat (Ehlers & Clark, 2000). Treatment therefore:

- **Identifies "stuck points"**, the beliefs about the trauma that keep it alive, such as "It was my fault," "I should have stopped it," or "The world is completely dangerous" (Resick et al., 2002).
- **Processes the memory** through exposure or a written account, so it becomes a memory of the past rather than a present danger (Ehlers & Clark, 2000).
- **Challenges and replaces** distorted blame and overgeneralized beliefs with balanced ones, and gradually drops avoidance and safety behaviors.

4. Religion, spirituality, and Christian integration: what the evidence shows

Faith can help or hurt. Across 49 studies, positive religious coping was linked to better adjustment to stress and negative religious coping (feeling punished or abandoned by God) to worse adjustment (Ano & Vasconcelles, 2005). Among 532 veterans in PTSD treatment, those higher on daily spiritual experiences, forgiveness, and positive religious coping fared better, while spiritual struggles predicted poorer outcomes (Currier et al., 2015).

Trauma can wound faith. In 1,385 veterans, killing others and failing to prevent death weakened religious faith, directly and through guilt, and weakened faith and guilt drove continued use of treatment more than symptom severity did (Fontana & Rosenheck, 2004). *Moral injury*, the lasting harm of perpetrating, failing to prevent, or witnessing acts that violate deeply held moral beliefs, has spiritual as well as psychological effects (Litz et al., 2009).

Intervention studies (small, but growing):

- **Christian-background women** (randomized, n = 43): an 11-session spiritually focused group for older survivors of abuse lowered depression, anxiety, and PTSD symptoms, with gains kept at 3 months (Bowland et al., 2012).
- **Building Spiritual Strength** (veterans): an 8-session spiritually integrated group reduced self-reported PTSD symptoms versus a wait-list (Harris et al., 2011). In a second trial (n = 138), it matched present-centered group therapy on clinician-rated PTSD and did better at easing distress in the relationship with God (Harris et al., 2018).
- **Spiritually Integrated CPT:** a 12-session adaptation of CPT that draws on clients' faith to address moral injury (Pearce et al., 2018). Evidence so far is a 6-person pilot showing improved PTSD symptoms and attachment to God (Duckworth et al., 2026).

Honest summary for teaching:

1. Trauma-focused CBT (CPT, cognitive therapy, prolonged exposure) is the evidence base. Faith integration supports it; it does not replace it.

2. Spiritual struggle and moral injury are common after trauma and predict worse outcomes, so assess them. Positive religious coping and forgiveness predict better outcomes.
3. Faith-integrated trauma treatments show promise in small trials. Spiritually integrated CPT fits Belief Counseling most closely but has not yet been tested in a full randomized trial.

5. Practical Christian-integration notes

These apply Dr. Campbell's identify, challenge, replace-with-Scripture method within trauma-focused CBT. Clinical items rest on the research above. Pastoral items are theological application, not research findings.

- **Identify (clinical).** Take a spiritual history: has the trauma strengthened or shaken faith? Listen for spiritual stuck points such as "God was punishing me," "God abandoned me," or "I can never be forgiven for what I did" (Currier et al., 2015; Fontana & Rosenheck, 2004).
- **Challenge (clinical).** Examine the evidence for distorted self-blame (what could the person actually know or control at the time?). Separate true moral responsibility, which calls for confession and repair, from false guilt, which calls for correction (Litz et al., 2009; Resick et al., 2002).
- **Replace (pastoral).** Offer Scripture of God's presence, healing, and forgiveness:
  - "He heals the brokenhearted and binds up their wounds." (Psalm 147:3)
  - "When you pass through the waters, I will be with you; and when you go through the rivers, they will not overwhelm you." (Isaiah 43:2). God was present in the trauma, not absent.
  - "Therefore, there is now no condemnation for those who are in Christ Jesus." (Romans 8:1). For false guilt and shame.
  - "If we confess our sins, He is faithful and just to forgive us our sins and to cleanse us from all unrighteousness." (1 John 1:9). For true moral injury.
- **Faith community (clinical).** Lack of social support predicts PTSD (Brewin et al., 2000). With consent, help the client reconnect with a supportive church, and invite a pastor or chaplain to help address moral injury.
- **Ministers:** offer presence and practical support; avoid pressing for detailed retelling or forced debriefing, and refer persistent symptoms beyond a month to a trained trauma therapist (Rose et al., 2002).

6. Quick classroom takeaways

- Diagnose by trauma exposure plus intrusion, avoidance, negative thinking and mood, and arousal lasting over a month. Screen with the PC-PTSD-5 and track with the PCL-5.
- Trauma-focused CBT (CPT, cognitive therapy, prolonged exposure) is first-line, with large effects; medication helps less. Do not use single-session debriefing.
- Assess spiritual struggle and moral injury. Faith-integrated trauma groups show promising early results; challenge distorted blame and replace it with the truth of God's presence and forgiveness.

Sources note

All 25 references were checked against Crossref and PubMed (Oct 31, 2026), and the figures quoted were checked against each study's abstract. Each DOI is a live link to the publisher record.

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